IGNIS CARE LIMITED
30 Pipit Walk Hemel Hempstead HP3 0FR

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Review Sheet

Last Reviewed 6 Mar 2026 | Last Amended 6 Mar 2026 | This policy will be reviewed as needs require or at the following interval: Annual

Business Impact:	MEDIUM — Changes are important, but urgent implementation is not required, incorporate into your existing workflow.
Reason for this Review:	Best practice
Changes Made:	Yes
Summary:	This policy will support staff with deprivation of liberty in community settings. It has been reviewed following further personalisation to present information about IGNIS CARE LIMITED, Values, Accessibility and Support Principles. For these changes to reflect in the policy the system details questionnaire will need to be updated.
Relevant Legislation:	• The Care Act 2014 • Care Quality Commission (Registration) Regulations 2009 • Human Rights Act 1998 • Mental Capacity Act 2005 • Mental Capacity Act Code of Practice • Deprivation of Liberty Safeguards Code of Practice (2008) • UK GDPR
Underpinning Knowledge:	• Author: NHSE, (2024), Identifying Restrictive Practice [Online] Available from: https://www.england.nhs.uk/long-read/identifying-restrictive-practice/ [Accessed: 06/03/2026] • Author: Social Care Wales, (2025), The Mental Capacity Act and Deprivation of Liberty Safeguards (DoLS) [Online] Available from: https://socialcare.wales/resources-guidance/social-care-workers/mental-capacity-act-dols [Accessed: 06/03/2026] • Author: ADASS, (2017), Quick Guide to Deprivation of Liberty Orders [Online] Available from: https://proceduresonline.com/trixcms2/media/17576/community-dol-a-quick-guide-to-community-dols.pdf [Accessed: 06/03/2026] • Author: Care Quality Commission, (2024), Outcome of an Application to Deprive a Person of their Liberty (DoLS) – Notification form [Online] Available from: https://www.cqc.org.uk/guidance-providers/notifications/application-deprive-person-their-liberty-dols-notification-form [Accessed: 06/03/2026] • Author: Supreme Court of the UK, (2014), Cheshire West and Cheshire Council v P [2014] UKSC 19 [Online] Available from:

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	https://www.mentalhealthlaw.co.uk/Cheshire_West_and_Chester_Council_v_P_(2014)_UKSC_19 [Accessed: 06/03/2026] • Author: DHSC, (2021), Liberty Protection Safeguards: What they are [Online] Available from: https://www.gov.uk/government/publications/liberty-protection-safeguards-factsheets/liberty-protection-safeguards-what-they-are [Accessed: 06/03/2026] • Author: GOV UK, (2018), Deprivation of Liberty Safeguards: Resources [Online] Available from: https://www.gov.uk/government/publications/deprivation-of-liberty-safeguards-forms-and-guidance [Accessed: 06/03/2026] • Author: CQC, (2022), Mental Capacity Act and Deprivation of Liberty Safeguards [Online] Available from: https://www.cqc.org.uk/guidance-providers/all-services/mental-capacity-act-deprivation-liberty-safeguards [Accessed: 06/03/2026]
Suggested Action:	• Encourage sharing the policy through the use of the QCS App • Share 'Key Facts' with all staff • Include discussion in staff handovers
Equality Impact Assessment:	QCS have undertaken an equality analysis during the review of this policy. This statement is a written record that demonstrates that we have shown due regard to the need to eliminate unlawful discrimination, advance equality of opportunity and foster good relations with respect to the characteristics protected by equality law.

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1. Purpose

1.1 About IGNIS CARE LIMITED

At Ignis Care, we aim to provide safe, stable, and person-centred support for people across a range of needs, enabling them to live fulfilling lives in the community.

Ignis Care delivers support for adults and children with **mental health conditions, learning disabilities and autism, dementia, sensory and physical disabilities**, and other complex needs. We work within **supported living and domiciliary care pathways**, always ensuring tenancy separation in line with the **REACH framework** and the Real Tenancy Test.

Our approach is rooted in **trauma-informed, values-based care**, with a focus on independence, recovery, and preventing crisis escalation. We employ skilled staff who are trained in Positive Behaviour Support (PBS), safeguarding, and specialist interventions, ensuring people are supported safely and compassionately.

Our ambition is to empower each person to achieve their personal goals — whether that means developing daily living skills, sustaining a tenancy, rebuilding relationships, returning to education or employment, or living safely with dignity in later life. We do this through tailored care planning, strong clinical governance, and close collaboration with families, commissioners, housing partners, and NHS/local authority professionals.

What are the values of IGNIS CARE LIMITED?

At Ignis Care, our values guide every decision, every shift, and every interaction. We are committed to:

- **Safety and Skilled Support** – providing high-quality, evidence-based care and support so that every person can achieve their optimum health, wellbeing, and independence.
- **Respect and Dignity** – treating every individual we support, every colleague, and every visitor with respect at all times, and recognising the uniqueness of each person.
- **Human Rights and Citizenship** – upholding the rights of every person to be included, to belong, and to have their voice heard in decisions that affect their life.
- **Choice and Independence** – encouraging informed choices and supporting decision-making, recognising that independence looks different for each person.
- **Privacy and Trust** – safeguarding personal information and respecting privacy in all aspects of daily life.
- **Personal Fulfilment** – enabling people to pursue meaningful goals, activities and relationships that give purpose and joy.
- **Continuous Learning and Improvement** – fostering a culture of reflection, training and feedback so that our staff grow and our service evolves.
- **Trauma-Informed, Person-Centred Care** – understanding people's stories and experiences, and shaping support that responds with compassion, patience and hope.

What are the Aims and Objectives of IGNIS CARE LIMITED?

1.2 To support IGNIS CARE LIMITED to protect the rights of people who might lack mental capacity who are aged 18 and above, when they may be deprived of their liberty in their

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best interests in their home environment.

They may lack capacity to consent to a Care Plan which:
- May deprive them of their liberty, but
- Is essential in order to give them necessary support/care or treatment

1.3 To comply with the Deprivation of Liberty Safeguards (DoLS) regulations in The Mental Capacity Act 2005 and to prevent breaches of the European Convention on Human Rights/Human Rights Act 1998.

1.4 To comply with the requirements of the European Convention Article/Human Rights Act 1998, 5(1)(e): '... No one shall be deprived of his liberty save in the following cases and in accordance with a procedure prescribed by law' and Article 5(4) 'Everyone who is deprived of his liberty... shall be entitled to take proceedings by which the lawfulness of his detention shall be decided speedily by a court and his release ordered if the detention is not lawful'.

1.5

Key Question	Quality Statements
CARING	QSC2: Treating people as individuals
CARING	QSC1: Kindness, compassion and dignity QSC3: Independence, choice and control
EFFECTIVE	QSE1: Assessing needs QSE2: Delivering evidence-based care & treatment
EFFECTIVE	QSE2: Delivering evidence-based care & treatment QSE3: How staff, teams & services work together
EFFECTIVE	QSE6: Consent to care and treatment

1.6 Relevant Legislation
- The Care Act 2014
- Care Quality Commission (Registration) Regulations 2009
- Human Rights Act 1998
- Mental Capacity Act 2005
- Mental Capacity Act Code of Practice
- Deprivation of Liberty Safeguards Code of Practice (2008)
- UK GDPR

2. Scope

2.1 Roles Affected:
- All Staff

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- Registered Manager
- Other management

2.2 People Affected:

- All adults Service Users who might lack mental capacity as defined under the Act in England and Wales
- Service Users aged 18+ who may lack mental capacity to consent to a care plan involving necessary restraint/restrictions that might amount to a DoL

2.3 Stakeholders Affected:

- Family
- Advocates
- Representatives
- Commissioners
- External health professionals
- Local Authority
- NHS
- Care Quality Commission
- GP
- Housing Providers and Landlords
- Staff where service users may be transferred to
- Legal Representatives
- Independent Mental Capacity Advocates (IMCAs)
- Relevant Person's Representatives (RPRs) appointed under DoLS
- ICB

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3. Objectives

3.1 Before any Service User is deprived of their liberty, all reasonable and practical efforts are made to avoid deprivation of liberty.

3.2 Service Users are not deprived of their liberty without lawful authority.

3.3 Authorisations by the Court of Protection to deprive Service Users of their liberty (including their duration and any conditions) are incorporated into Care Plans, and full information is given to the Service User and their relatives or friends who are interested in their welfare.

Appropriate notifications are made to the Care Quality Commission (CQC).

3.4 If a person lacks capacity to consent, all Support interventions are carried out in agreement with the wider Mental Capacity Act (MCA). The Support must be in the person's best interests, with actions taken in a proportionate way using the least restrictive options possible.

3.5 Support Workers are trained and work within the framework of the Mental Capacity Act 2005.

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3.6 All staff understand their responsibilities under the Mental Capacity Act, including Deprivation of Liberty Safeguards (MCA DoLS) and Codes of Practice.

4. Policy

4.1 The Registered Manager, Mrs Miriam Chengeto Cummings, and Nominated Individual, Daniel Monem, of IGNIS CARE LIMITED, have overall management responsibility for this policy and procedure. This is in line with the Policy Management Policy and Procedure at IGNIS CARE LIMITED.

4.2 Mental Capacity Act 2005

IGNIS CARE LIMITED works within the Mental Capacity Act 2005 including Deprivation of Liberty Safeguards (MCA DoLS) and its codes of practice, and follows guidance about what amounts to deprivation of liberty given in the 'Cheshire West Supreme Court Judgement' CQC briefing.

This policy should be read alongside the Mental Capacity Act (MCA) 2005 Policy and Procedure and the Restrictive Practices Including Restraint and Physical Interventions Policy and Procedure of IGNIS CARE LIMITED, which outline information on the Act including the 5 Principles, best interest decisions, restrictive practices, physical intervention and restraint.

4.3 IGNIS CARE LIMITED Statement

IGNIS CARE LIMITED supports a person-centred approach to Support and upholds the rights of Service Users outlined within the Human Rights Act.

IGNIS CARE LIMITED will use every means available to it to maximise the mental capacity of Service Users to make their own decisions, in accordance with the Mental Capacity Act 2005 and its codes of practice.

IGNIS CARE LIMITED will ensure information is accessible to all stakeholders when talking about deprivation of liberties and will ensure the wishes and feelings of Service Users and their representatives are heard.

4.4 Authorisation to Deprive a Service User of their Liberty in a Community Setting

IGNIS CARE LIMITED understands that sometimes a legal decision is made to deprive a Service User in a community setting of their liberties when all alternatives have been exhausted. IGNIS CARE LIMITED cannot legally, and will not, deprive Service Users of their liberties without the correct authorisation from the Court of Protection.

4.5 Support of a New Service User

IGNIS CARE LIMITED will not take on the Support of a Service User if it suspects the Service User is being deprived of their liberties without the correct authorisation in place, until they are in place.

IGNIS CARE LIMITED will communicate with commissioners who are responsible for seeking authorisation from the Court of Protection.

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4.6 Wrongful Deprivation of Liberties

Where IGNIS CARE LIMITED assesses a Service User as being wrongfully deprived of their liberty without authorisation, IGNIS CARE LIMITED will continue to provide a Support service to the Service User, making sure their liberty is no longer restricted.

4.7 Monitoring

The Registered Manager will monitor the use of this policy and will make any updates necessary to its correct application.

Any updates to guidance and legislation will be reflected in this policy.

4.8 Training

All staff will be given training in the Mental Capacity Act 2005 and its Code of Practice. Training will be tailored provided that is appropriate to staff members' roles and their responsibilities to Service Users in relation to informed consent, best interest decisions, restrictive practices, physical interventions/restraint and deprivation of liberty.

4.9 Liberty Protection Safeguards

IGNIS CARE LIMITED acknowledges the future introduction of the Liberty Protection Safeguards. At this point, this policy and procedure will be revised to reflect those changes.

The Liberty Protection Safeguards were introduced in the Mental Capacity (Amendment) Act 2019 and will replace the Deprivation of Liberty Safeguards (DoLS) system. The Liberty Protection Safeguards will deliver improved outcomes for people who are or who need to be deprived of their liberty.

4.10 Policy Accessibility

IGNIS CARE LIMITED understands that some Service Users may take in and retain information in different ways. To support full understanding and engagement, this policy is available in accessible formats. The ReciteMe tool has various ways of making this policy accessible by providing this policy in:

- Audio
- Large print
- Multiple languages

This policy can also be made available in:

- Easy-read versions
- Simple-policy view to reduce navigation and complexity

These options are in place to help Service Users to understand and engage with this policy more easily.

5. Procedure

5.1 Roles and responsibilities

The **Registered Manager** is responsible for:

- This policy and the dissemination of it
- All related training and guidance

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- Liaising with commissioners regarding their responsibility to apply to the Court of Protection for authorisation for the deprivation of liberty of a Service User
- Making capacity assessments when they are needed
- Coordinating best interest decisions at IGNIS CARE LIMITED

All staff have a duty to:

- Read this policy and procedure
- Raise any related questions or queries with their line manager
- Work within the parameters of this policy and procedure
- Report any concerns of the mistreatment or wrongful detention of a Service User to the Registered Manager

Staff can use the Raising Concerns, Freedom to Speak Up and Whistleblowing Policy and Procedure at IGNIS CARE LIMITED where they wish to use its protections.

Any member of staff deliberately preventing another member of staff from reporting concerns will be subject to the Discipline Policy and Procedure.

5.2 Capacity

Service Users should be given the information they need in the way that they can understand, to give informed consent to all aspects of their Support. Where staff have reason to believe a Service User lacks capacity, they should record the information and contact the Registered Manager who will decide if a capacity assessment is needed. Details of the capacity assessment can be found in the Mental Capacity Act (MCA) 2005 Policy and Procedure.

5.3 Deprivation of Liberty

When carrying out a capacity assessment, if the Registered Manager has carried out the assessment and deprivation of liberty appears necessary and proportionate, and unavoidable after exhausting all other possibilities, they must notify the Supervisory Body and commissioners and ask them to apply appropriately and in a timely way to the Court of Protection for authorisation.

A checklist of questions to help determine deprivation of liberty can be found in the Forms section of this policy.

5.4 Accessible Information

Service Users and their representatives should be communicated with in a way that is understood as far as possible, about the potential deprivation of liberty and next steps. Communication should be timely and ongoing, and their wishes and feelings should be recorded and shared with commissioners and the Court.

5.5 Record

The Registered Manager will keep full records of best interest decisions, assessments, applications, discussions with the Service User and their relatives/friends about deprivation of liberty and the actions taken to minimise its use.

5.6 Commissioner Information

The Registered Manager will ensure that all relevant information needed for discussion with the commissioners is ready - a list of which can be found in the Forms section of this policy.

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The commissioning body is responsible for submitting the application to the Court of Protection with the supporting information from IGNIS CARE LIMITED.

5.7 If Deprivation of Liberty is Authorised by the Court of Protection:

The Registered Manager will:

- Ensure that the Service User and their relatives understand what restrictions have been authorised, and how they can challenge any aspect of the authorisation with the help of an IMCA (Independent Mental Capacity Advocate)
- Record the end date set by the Court of Protection and arrange, at least a month earlier, to convene a meeting to have discussions with commissioners or the Local Authority about whether a new authorisation will be needed
- Notify the CQC of the Court of Protection application and its outcome
- Record any conditions that are included in the authorisation and ensure that they are incorporated into the Care Plan making sure that all staff are aware of these conditions

5.8 Court of Protection Appointed Assessors Visit

The Registered Manager will ensure the availability of the Service User and staff as well as the Care Plan and assessments for the Court of Protection Appointed Assessors to visit the service, who will interview the Service User, view records, and may interview staff.

5.9 Wrongful Deprivation of Liberty

Where the Registered Manager assesses the wrongful deprivation of liberty of a Service User, they will ensure that the restrictions are lifted, and the Care Plan is reviewed to ensure it reflects the changes to practice.

Support Workers at IGNIS CARE LIMITED will be supported in their understanding and learning, where this is appropriate, through their line manager.

The Registered Manager will contact the relevant commissioning body, the Daniel Monem and the Police where a crime is suspected.

5.10 Care and Support at IGNIS CARE LIMITED

IGNIS CARE LIMITED provides care and support to Service Users with a range of needs in a person-centred, safe, and lawful way.

All staff must follow the guidance within this policy and the Service User's Care Plan, ensuring that assessed needs, reasonable adjustments and individual preferences are met.

- Every Service User is treated equally and with dignity and respect
- Care and support are tailored to individual needs, preferences and desired outcomes
- Staff follow legal, regulatory, and professional guidance at all times
- Person-centred approaches are used to promote independence, choice, and wellbeing

To support this approach, staff will also follow the policies and procedures below where applicable:

- Person-Centred Care and Supporting Planning
- Safeguarding Adults
- Raising Concerns, Freedom to Speak Up and Whistleblowing

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Daniel Monem

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- Mental Capacity Act (MCA) 2005
- Deprivation Of Liberty in Community Settings
- Consent to Care, Support and Treatment
- Equality, Diversity and Human Rights
- Overarching Medicines Management
- Positive Behaviour Support Including Challenging Behaviour
- Restrictive Practices Including Restraint and Physical Interventions
- Sex, Sexuality and Relationships

This list is not exhaustive and there will be additional policies and procedures in place to support specific Service User needs. Staff must seek clarification from their line manager or the Registered Manager, Mrs Miriam Chengeto Cummings, if there is any uncertainty.

Staff supporting any specialist area of need will receive appropriate induction and training. They will complete competency assessments, where required, to meet the needs of Service Users as outlined in the Training Policy and Procedure at IGNIS CARE LIMITED.

To ensure Service Users receive person-centred care and support from IGNIS CARE LIMITED, staff must adhere to the following :

6. Definitions

6.1 Deprivation of Liberty 'Acid Test'

- The Supreme Court 'Acid Test' clarifies that a person is deprived of their liberty if they:
  - Lack capacity to consent to the arrangements needed to give them necessary care, support or treatment
  - Are not free to leave (they may be allowed to go out with staff, or even alone, with permission, but may not go to live somewhere else, or without staff permission), and
  - Are subject to continuous (or continual) supervision and control. (A way to think about this is, do staff know all the time where someone is and what they are doing, and do staff provide and control their access to other people, activities, and the community?)
  - **Note:** DoLS can only be used for people aged 18 or over, in care homes or hospitals. For people aged 16 or 17, or living in other settings, the commissioners must apply to the Court of Protection for authority to deprive them of liberty

6.2 Court of Protection Authorisation Conditions

- There are not always conditions attached to an authorisation, but where they are imposed, the intention is to lessen the deprivation of liberty in some way; for example, by saying the provider must arrange taking the person out into the community regularly, or do their best to make it possible for relatives or friends to visit the person and, where possible and safe, take them out
- Conditions are part of the legal authorisation so must be complied with. If a provider cannot comply with any condition, they should urgently contact the commissioners of the service to discuss the best way to proceed

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Delegated: 15 March 2026
Daniel Monem

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6.3 Restraint

- As defined by the MCA, this is the use, or threat, of force to make someone lacking mental capacity do something they are resisting, or restricting the freedom of movement of someone lacking mental capacity, whether the person resists this or not
- Restraint is only lawful if it is in the person's best interests: except in an emergency, best interests are worked out in accordance with the statutory checklist in MCA Section 4 (See MCA code of practice Chapter 5). But restraint, to be lawful, must also meet two extra conditions (see MCA code of practice 6.40 and following)
- The restraint must be necessary to prevent harm to the person, and a proportionate response to the likelihood and seriousness of that harm; its intensity and duration must be as minimal as possible
- Restraint is considered part of a deprivation of liberty but individual instances of restraint do not themselves constitute a deprivation of liberty; this is because deprivation of liberty is defined by reference to the entire Care Plan, and how it meets the 'Acid Test' (see Deprivation of Liberty 'Acid Test' definition)

6.4 Independent Mental Capacity Advocate (IMCA) Role in DoLS

- The Supervisory Body will appoint an IMCA to represent someone who is subject to an authorisation request if they have no relatives or friends to be consulted as part of the process
- The IMCA will arrange with Care Plan or the hospital if they are going to visit. They have the right to see relevant notes and to meet with the person alone if they wish
- An IMCA may be appointed after an authorisation is granted to help the Relevant Person's Representative (RPR) understand the process if they wish to help the person to challenge their authorisation. They will only be appointed to help unpaid RPRs, who are usually relatives or friends, not paid RPRs who are usually paid advocates

6.5 Community Services

- This policy refers to such services as supported living, shared lives schemes, extra-care housing and domiciliary care services, providing services to people aged 16 and over, who lack capacity to consent to arrangements for giving them necessary care or treatment, when those arrangements may amount to a deprivation of liberty

6.6 Liberty Protection Safeguards (LPS) - What they are

- DoLS, which only protect the rights of people lacking capacity in care homes or hospitals, and only from 18 upwards, will be replaced, at a date to be confirmed, by the LPS
- LPS will protect the rights not only of this group of people, but of anyone aged from 16 upwards, in any setting, for example, supported living, extra-care housing, or their own family homes, who needs to be deprived of their liberty in their best interests
- Like DoLS, the purpose of the LPS is to protect the rights of people who are restricted or restrained in their best interests. However, the LPS process will be simpler than DoLS. It will ensure that the person's wishes and feelings are at the heart of all arrangements for their Support, and that any restrictions on their freedom are **necessary** to prevent harm to the person, and a **proportionate** response to how likely that harm is, and how serious it would be
- More information on LPS, including the implementation date, guidance and learning resources, will be available here and on the QCS website in good time to prepare for implementation

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6.7 Protections Given by DoLS

- The DoLS assessments ensure that independent professionals examine the decision-making that has led to the person being deprived of their liberty
- The assessors check that the person does lack capacity to make relevant decisions, and that they do have a mental disorder
- The best interests assessor (BIA) also checks that the arrangements are in the person's best interests, and both necessary to prevent harm to the person and a proportionate response to the likelihood and seriousness of such harm
- An authorisation allows the person, or someone acting on their behalf, to challenge any part of their authorisation, to the local authority or to the Court of Protection

6.8 DoLS Reviews

- Anyone can ask for the authorisation to be reviewed by the Supervisory Body (DoLS team)
- This is usually because there is doubt about whether the person might have regained capacity, or if the arrangements are no longer in their best interests for some reason, or no longer proportionate

7. Key Facts - Professionals

Professionals providing this service should be aware of the following:

- Deprivation of liberty is governed under the MCA, so all recording must show compliance with the wider MCA, including the search for the least restrictive option to deliver services
- Deprivation of liberty is sometimes a necessary and proportionate way to keep a person who lacks mental capacity safe
- The risk of deprivation of liberty must be recognised and its use removed, or reduced as far as possible, while preventing foreseeable harm to the person lacking mental capacity
- Its use is not lawful without an authorisation having been sought from the appropriate body which, for community services such as supported living, extra-care housing, shared lives and domiciliary care, or where the person is aged 16 or 17, is the Court of Protection
- An authorisation is only granted if the Court agrees that there are no less-restrictive options to keep the person safe
- Authorisation provides protection from liability for staff who are depriving the person of their liberty
- The outcome of an Authorisation Request must be notified to the Care Quality Commission

8. Key Facts - People Affected by The Service

People affected by this service should be aware of the following:

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- An authorisation by the Court of Protection for someone to be deprived of their liberty is a way to protect the rights of people cared for in their own homes
- An authorisation provides reassurance that the care is in the best interests of the person, and that no less restrictive options can be found to keep the person safe
- Authorisation allows the person (or their representative) or anyone with a concern to appeal to the Court of Protection for a decision about their mental capacity to decide how to live, or whether the authorisation is in their best interests and the least restrictive option for their care
- There is no stigma attached to an authorisation: it is a sign that the provider is acting to protect the human rights of people who are vulnerable

Further Reading

GOV.UK - Liberty Protection Safeguards factsheets: LPS will replace DoLS and are planned to come into force at a later date. For up-to-date information and factsheets, see:

https://www.gov.uk/government/publications/liberty-protection-safeguards-factsheets

Care Quality Commission - Health and Social Care Act 2008 (Regulated Activities) Regulations 2014: Regulation 11 - Need for consent:

https://www.cqc.org.uk/guidance-providers/regulations-enforcement/regulation-11-need-consent

Care Quality Commission - Health and Social Care Act 2008 (Regulated Activities) Regulations 2014: Regulation 13 - Safeguarding service users from abuse and improper treatment:

https://www.cqc.org.uk/guidance-providers/regulations-enforcement/regulation-13-safeguarding-service-users-abuse-improper

Community Care - UPDATED: October 2023 date for Liberty Protection Safeguards 'incorrect':

https://www.communitycare.co.uk/2022/10/28/liberty-protection-safeguards-due-for-implementation-in-october-2023/

Outstanding Practice

To be "outstanding" in this policy area you could provide evidence that:

- All Care Plans are created with the wishes and feelings of Service Users at their centre; their wishes and feelings are accommodated wherever this can be done
- Deprivation of liberty is avoided wherever possible by the use of creative and person-centred strategies

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- Deprivation of Liberty Authorisation is sought in a timely way from the Court of Protection, and Ignis Care Limited supports the Service User through the process
- Relevant dates and any conditions of authorisations are properly recorded; conditions are met, and new authorisations are sought in a timely fashion
- The wide understanding of the policy is enabled by proactive use of the QCS App
- All Care Plans for Service Users lacking mental capacity benefit from the knowledge and insight of the Service User's relatives and friends in knowing how to meet the Service User's past and present wishes and feelings
- People subject to authorisation and their relatives, friends or representatives are given full information about authorisation requests and all information that is prescribed in law

Forms

The following forms are included as part of this policy:

Title of form	When would the form be used?	Created by
Deprivation of Liberties Questions - CR83	To consider whether a service user who lacks Mental Capacity is being unlawfully deprived of their liberties	QCS
Checklist of Information to Commissioners/for Application to Court of Protection - CR83	When speaking to the commissioning body/making an application to the Court of Protection.	QCS

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Deprivation of Liberties Questions - CR83

The Supreme Court has now confirmed that there are two key questions to ask when considering if a Service User is being deprived of their liberties (marked 2 and 3).	
1. Is the Service User 16 or over and have they been assessed as lacking capacity to the Support arrangements?	**Yes / No**
2. Is the person subject to continuous supervision and control?	**Yes / No (Give further details)**
3. Is the person free to leave? (The person may seem happy to stay, but the issue is about how staff would react if the person did try to leave)	**Yes / No (Give further details)**

Consider the answers to these questions, the Service User may be being deprived of their liberties if there are measures or restrictions in place that limit their choice and consent.

Is there a Court of Protection Authorisation to deprive the Service User of their liberties?	**Yes / No**

If 'NO', complete the Checklist of Information to Commissioners and contact the commissioning body (Local Authority or NHS) to discuss the situation, adjust the Care

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Plan to stop the deprivation of liberties, OR submit an application to the Court of Protection for Authorisation of Deprivation of Liberties.

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IGNIS CARE LIMITED
30 Pipit Walk Hemel Hempstead HP3 0FR

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Checklist of Information to Commissioners/for Application to Court of Protection - CR83

The information below should be at hand to discuss with the commissioning body.	
Information/Documents	**Have to hand (tick)**
Any supporting medical evidence to show the Service User lacks the capacity to consent to the Support arrangements.	
A dated copy of the Service User's Care Plan.	
Does the Care Plan outline why it is said that the Service User does require or may require a deprivation of liberty?	
Copy of best interests meeting notes.	
Any relevant Advance Decisions or any relevant decisions under a Lasting Power of Attorney.	
Any information relevant to the case.	
Wishes and feelings of the Service User/relevant persons.	

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